

Supporting Information

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S Fig S1. Correlation between simulated ventricular total activation time (TAT_V) and recorded ECG QRS duration, stratified by condition. In *C* hearts (blue, $n = 17$), TAT_V showed a strong positive correlation with QRS duration (Pearson $r = 0.649$, $p = 0.005$), indicating that anatomical variability alone can predict conduction time in healthy myocardium when using uniform electrophysiological parameters. In *HF* patients (red, $n = 16$), no correlation was observed ($r = -0.127$, $p = 0.64$), reflecting that pathological conduction delays (fibrosis, scar, conduction blocks) are not captured by anatomy alone and would require patient-specific tissue property calibration.

S Table S1. Geometric characteristics stratified by sex and heart failure status. Mean $\pm$ SD reported for age (years), chamber volumes (mL), and total mesh elements (millions). *HF* patients showed increased chamber volumes across all chambers, particularly pronounced in males. Mesh element counts scaled with anatomical size, reflecting the adaptive meshing algorithm.

Group	n	Age (yr)	Vol _{LV} (mL)	Vol _{RV} (mL)	Vol _{LA} (mL)	Vol _{RA} (mL)	Elements ($\times 10^6$)
Male <i>C</i>	13	47.2 $\pm$ 12.0	179.6 $\pm$ 36.3	188.7 $\pm$ 25.7	93.9 $\pm$ 27.9	100.7 $\pm$ 16.9	2.33 $\pm$ 0.35
Male <i>HF</i>	12	60.1 $\pm$ 16.1	289.1 $\pm$ 88.1	227.6 $\pm$ 60.2	140.5 $\pm$ 58.9	138.2 $\pm$ 67.7	2.95 $\pm$ 0.59
Female <i>C</i>	13	49.5 $\pm$ 9.8	139.7 $\pm$ 32.2	139.7 $\pm$ 34.8	79.6 $\pm$ 19.4	86.3 $\pm$ 32.3	1.91 $\pm$ 0.28
Female <i>HF</i>	12	60.1 $\pm$ 9.0	183.0 $\pm$ 60.0	136.7 $\pm$ 30.8	104.9 $\pm$ 57.4	100.0 $\pm$ 47.3	2.17 $\pm$ 0.38

S Fig S2. Correlation matrix between geometric variables and simulation outputs. Spearman correlation coefficients (ρ) shown for relationships between geometric metrics (chamber volumes, mesh element count, age) and simulation outputs (activation times, volume changes). Strong positive correlations (red) indicate that geometric variables predict functional outcomes under standardised parameters. All correlations shown are statistically significant ($p < 0.05$).

CGAL Meshing Parameters

Mesh density is controlled via parameters of the Computational Geometry Algorithms Library (CGAL) [1], specified globally for the entire heart geometry. Regional refinement was not applied; a single set of parameters governs element density and surface fidelity across all anatomical structures.

Mesh Label Specification

The post-processing stage of the CEMRG Heartbuilder pipeline increases the initial 10-label segmentation to 37 labels (Figure ??c), following the standard introduced by [2, 3]. Labels encode anatomical structures required for assigning tissue-specific electrophysiology and mechanics model parameters during simulation setup.

Mechanics Simulation Boundary Conditions

As boundary conditions, the pericardial sac was modelled using normal springs with varying stiffness and omni-directional springs were applied at the superior vena cava and the right pulmonary veins [4]. All the tissues were modelled as incompressible with an incompressibility value of $\kappa = 1000$ kPa. The bulk stiffness of left ventricle, right ventricle and atria were set to $a_{LV} = 1.7$ kPa, $a_{RV} = 2.55$ kPa, and $a_{atria} = 3.4$ kPa, respectively. The anisotropy factors in the ventricles in the fibre, cross-fibre and cross-sheet directions were

S Table S2. CGAL meshing parameters used for volumetric mesh generation. Parameters are specified globally for the entire heart geometry. `cell_size` and `facet_size` set the target element edge length; `facet_distance` controls the surface approximation error (trade-off between surface fidelity and smoothness). All 50 meshes were generated with identical parameter values.

Parameter	Value	Effect on mesh
<code>cell_size</code>	≈ 0.5 mm	Maximum target edge length of volumetric tetrahedral elements; controls overall mesh density.
<code>facet_size</code>	≈ 0.5 mm	Maximum target size of surface facets; controls surface mesh resolution and matches <code>cell_size</code> for uniform density.
<code>facet_distance</code>	<i>(add value)</i>	Maximum Hausdorff distance between the input surface and the Delaunay surface; Laplacian smoothing is applied to balance surface fidelity against smoothness.
<code>cell_radius_edge_ratio</code>	<i>(add value if used)</i>	Upper bound on the ratio of circumradius to shortest edge; controls element shape quality.

S Table S3. Complete list of 37 anatomical labels used in mesh generation. Labels are assigned during the post-processing stage of the segmentation pipeline. Structure types: BP = blood pool, Myo = myocardium, VP = valve plane, Ring = vein/artery ring.

Label ID	Structure name	Chamber	Type
<i>(add)</i>	Left ventricular blood pool	LV	BP
<i>(add)</i>	Left ventricular myocardium	LV	Myo
<i>(add)</i>	Right ventricular blood pool	RV	BP
<i>(add)</i>	Right ventricular myocardium	RV	Myo
<i>(add)</i>	Left atrial blood pool	LA	BP
<i>(add)</i>	Left atrial myocardium	LA	Myo
<i>(add)</i>	Left atrial appendage	LA	BP
<i>(add)</i>	Right atrial blood pool	RA	BP
<i>(add)</i>	Right atrial myocardium	RA	Myo
<i>(add)</i>	Aortic blood pool	—	BP
<i>(add)</i>	Aortic wall	—	Myo
<i>(add)</i>	Pulmonary artery blood pool	—	BP
<i>(add)</i>	Pulmonary artery wall	—	Myo
<i>(add)</i>	Left superior pulmonary vein	LA	BP
<i>(add)</i>	Left inferior pulmonary vein	LA	BP
<i>(add)</i>	Right superior pulmonary vein	RA	BP
<i>(add)</i>	Right inferior pulmonary vein	RA	BP
<i>(add)</i>	Superior vena cava	RA	BP
<i>(add)</i>	Inferior vena cava	RA	BP
<i>(add)</i>	Mitral valve plane	LV/LA	VP
<i>(add)</i>	Tricuspid valve plane	RV/RA	VP
<i>(add)</i>	Aortic valve plane	LV	VP
<i>(add)</i>	Pulmonary valve plane	RV	VP

set to $b_f = 8$, $b_{fs} = 4$, and $b_t = 3$, respectively; while in the atria were set to $b_f = 16$, $b_{fs} = 8$, and $b_t = 6$. The valves, aorta, pulmonary artery and vein rings had a stiffness value of $c_{\text{valves}} = 1000$ kPa, $c_{\text{aorta}} = 26.66$ kPa, $c_{\text{PA}} = 3.7$ kPa, and $c_{\text{rings}} = 7.45$ kPa, respectively. The maximum stiffness at the pericardium was set to 0.005 kPa/ μm and to 0.001 kPa/ μm in the veins. The pressure was increased linearly from 0 to 1 mmHg in increments of 0.02 mmHg/ms.

References

1. CGAL 4.11 - Manual: Package Overview;. Available from: <https://doc.cgal.org/4.11/Manual/packages.html>.
2. Strocchi M, Augustin CM, Gsell MAF, Karabelas E, Neic A, Gillette K, et al. A publicly available virtual cohort of four-chamber heart meshes for cardiac electro-mechanics simulations. PLOS ONE. 2020;15(6):e0235145. Publisher: Public Library of Science. doi:10.1371/journal.pone.0235145.
3. Strocchi M, Gsell MAF, Augustin CM, Razeghi O, Roney CH, Prassl AJ, et al. Simulating ventricular systolic motion in a four-chamber heart model with spatially varying robin boundary conditions to model the effect of the pericardium. Journal of Biomechanics. 2020 Mar;101:109645. doi:10.1016/j.jbiomech.2020.109645.
4. Strocchi M, Longobardi S, Augustin CM, Gsell MAF, Petras A, Rinaldi CA, et al. Cell to whole organ global sensitivity analysis on a four-chamber heart electromechanics model using Gaussian processes emulators. PLOS Computational Biology. 2023 Jun;19(6):e1011257. Publisher: Public Library of Science. doi:10.1371/journal.pcbi.1011257.